

OrthoSurplus and Dr Bola Akinola

Three distinct brands, one commercial arrangement: how OrthoSurplus and Dr Bola work together to build affordable arthroplasty at scale

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How to read this

An earlier draft of this strategy blurred three separate brands into a single group. This version corrects that. The brands stay distinct; what connects them is a commercial arrangement, not a merger. This document sets out the three brands, the arrangement between OrthoSurplus and Dr Bola, and why it is powerful for both. It builds on the Blue Ocean plan already delivered for Dr Bola.

It is greenfield: there was no prior written OrthoSurplus strategy in our files, so this is drawn from the venture as it stands, the market work behind the Blue Ocean plan, and the vision for a roaming, hospital-partnered arthroplasty capability with Dr Bola as lead clinical anchor. Numbers and terms in [brackets] are to confirm.

1. Three brands, and who owns what

Keeping these separate is the whole point, so state it plainly.

Brand	Whose it is	What it is	Role here
Dr Bola Akinola	His (primary personal brand)	The UK-trained surgeon; complex-case and revision authority; the clinical trust and the face	The clinical engine of the arrangement
Osteon Clinics	His (secondary; he owns it)	His clinic and institution, where he personally delivers care	One delivery site and flagship, stays his
OrthoSurplus	C4A's (independent venture)	The implant-supply platform, and the arthroplasty-delivery capability being built around it	The platform Dr Bola contracts with

Dr Bola and Osteon are his own brands, unchanged by any of this. **OrthoSurplus is a separate C4A brand that enters a commercial arrangement with him.** It does not own him, his practice or his clinic. That independence is a feature, not a limitation, and Section 6 explains why it matters.

2. The idea in one line

OrthoSurplus already controls the biggest cost in a joint replacement: the implant, 40 to 60 percent of a \$5 to 15 million bill. Dr Bola is the scarce, excellent surgeon, with the training to make more. **A commercial arrangement between the two lets OrthoSurplus build an affordable, credible, scalable arthroplasty capability, and gives Dr Bola a cost advantage and a platform he could never build alone, while both keep their own brands.**

Neither has to become the other. They simply agree to work together, each doing what it is best at.

3. What OrthoSurplus is building (its own venture)

This is OrthoSurplus's platform, not a joint entity. It has four parts, and Dr Bola is engaged commercially to power the clinical ones.

Layer	What it is	Owner
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Supply	Certified implants at 40 to 60 percent of cost, transparent pricing, managed sterilised kits	OrthoSurplus
Talent	An academy that trains associate arthroplasty surgeons	OrthoSurplus, taught by Dr Bola (and Dr Kumar) under contract
Delivery	A roaming arthroplasty unit that brings surgeon, implants, kit and protocols to the hospital	OrthoSurplus, led clinically by Dr Bola
Distribution	A packaged arthroplasty service line sold to hospitals that lack one	OrthoSurplus, delivered by its clinical anchors

Dr Bola is the lead clinical anchor across the talent, delivery and distribution layers, under a defined commercial arrangement. Dr Kumar (Abuja) is intended as a second anchor but is currently focused on his own new practice, so early on this runs on Dr Bola. Building the academy cadre early is how OrthoSurplus avoids being one surgeon deep.

4. The packaged hospital offer (the commercial heart)

This is the product that scales OrthoSurplus: **be the arthroplasty partner for hospitals that cannot run arthroplasty themselves**. Most Nigerian hospitals have beds, theatres and patients with worn-out joints, but no arthroplasty surgeon, no reliable implant supply, and no protocols. They lose those patients elsewhere or abroad. OrthoSurplus hands them a service line in a box.

What the hospital gets: a visiting anchor or trained associate surgeon, the certified implants, the sterilised instrument kits, the peri-operative protocols, training for its own theatre and ward staff, and a published-outcomes standard. A joint-replacement service switched on, with none of the capital or hiring risk of building it alone.

What OrthoSurplus gets: guaranteed theatre access and a pipeline of cases without owning real estate, implant sales, and a widening network of hospital spokes. The hospital keeps its facility revenue (bed, theatre, nursing, imaging, aftercare); OrthoSurplus earns on the surgical episode and the implant. Added volume for the hospital, not lost business, which makes it an easy yes.

What Dr Bola gets (through the arrangement): paid clinical work, a platform to reach far more patients than his own clinic can, and the affordable implant cost that makes his Blue Ocean fixed-price positioning real.

Commercial shape (to model): a per-case managed-service fee or a revenue share on the surgical episode for OrthoSurplus, the implant margin, and a defined clinical fee or share to Dr Bola for the cases he leads, with a clear split of who owns aftercare and clinical governance.

5. The roaming arthroplasty unit

OrthoSurplus's delivery mechanism, and the practical form of the hub-and-spoke model Dr Bola already runs informally at Cedarcrest and Diamed. A standing, mobile capability: a lead surgeon (Dr Bola to begin, trained associates as the academy grows), the implants and managed kits, a defined peri-operative pathway, and a protocol for handing recovery back to the partner hospital's team.

Why it fits:

- It matches supply to where demand actually is, rather than forcing patients to a single premium clinic.
- It creates the volume a focused factory needs for both cost and quality (surgeons who do many cases get better and cheaper, the Narayana lesson).



- For Dr Bola specifically, it dissolves his location problem entirely: the unit roams, hospitals are the spokes, and Amuwo Odofin stops mattering. We are formalising and productising what he does anyway, inside OrthoSurplus rather than his own practice.

The governance is real and designed in: consistent standards across sites, clear ownership of complications away from base, a clean aftercare handoff. Section 7 covers it.

6. How the three brands reinforce each other

The value is in the arrangement, and it runs both ways, without anyone losing their brand.

Dr Bola makes OrthoSurplus credible. The biggest objection to lower-cost implants is "are they any good?" A respected FRCS arthroplasty surgeon who uses them and publishes his outcomes by implant answers that in the only way that counts. He turns a price story into a trust story.

OrthoSurplus gives Dr Bola his moat. The affordable, fixed-price, transparent arthroplasty his Blue Ocean plan depends on is only possible if the implant cost is controlled. Buying through OrthoSurplus is that control, and it is the one advantage a better-funded competitor paying full implant markup cannot easily match.

His referral network is OrthoSurplus's market, and vice versa. The surgeons who send Dr Bola their complex cases are the surgeons OrthoSurplus sells implants to, the pool the academy recruits from, and the doctors at partner hospitals. One set of relationships, several revenue lines, shared without merging anyone.

Osteon stays his flagship. His own clinic remains the place he personally delivers and the proof site for his outcomes. It is a node in the network, on his terms, not absorbed by it.

That is a two-sided flywheel: cheaper implants plus a trusted surgeon lower price and raise trust, which brings patients and hospitals, which brings volume, which lowers cost and trains more surgeons, which widens the network. Each brand keeps its identity and gains from the others.

7. The conflict-of-interest question, handled head-on

Because OrthoSurplus is an independent C4A brand and not Dr Bola's own business, the sharpest version of this problem does not arise: he is not selling his own implants. He is a clinician in a disclosed commercial arrangement with an arm's-length supplier, which is ordinary and defensible. It still must be handled cleanly, because his whole brand is honesty.

- **Full, upfront disclosure** to patients and partner hospitals of the arrangement between him and OrthoSurplus. Say it plainly.
- **A clinical-governance firewall:** implant selection driven by clinical merit and value for the patient, on published criteria, never by anyone's margin. Ideally an independent clinical voice signs off the formulary.
- **The framing is the mission, not the markup.** The honest story is Aurolab's (the Aravind lens that went from \$70 to \$2): a supplier brought the implant cost down so patients could afford the operation, and a surgeon chose to work with it for that reason. Told straight, disclosure plus lower prices plus published outcomes is more trustworthy than a rival who hides his margins entirely.

Handled this way, the arrangement reads as exactly what it is: a surgeon working with an affordable-supply platform to widen access.

8. Why this makes the competition irrelevant

The Blue Ocean plan argued Dr Bola should create uncontested space rather than fight on the industry's terms. This arrangement is how that space gets built at scale, and it is OrthoSurplus that carries it.



- **It targets the noncustomers, not Oni's patients:** the thousands priced out, the hospitals with no arthroplasty, the patients who fly abroad or suffer on. A vertically integrated, roaming, affordable model reaches all three tiers; a single premium Lagos clinic reaches none of them.
- **It competes on a different axis:** access, transparency and affordability, delivered as infrastructure. You cannot out-luxury your way to answering it, and you cannot copy it without building the same supply, cadre and network.
- **It is defensible:** the implant cost advantage, the trained cadre, the hospital network and the outcomes data all strengthen with scale.

9. What each party gets

- **OrthoSurplus:** clinical credibility, a delivery and distribution engine on top of pure supply, and a defensible, scalable arthroplasty platform.
- **Dr Bola:** a cost advantage that makes his affordable positioning real, paid clinical work far beyond his own clinic's reach, a platform that scales his expertise through training and roaming, and a mission that fits his ethics-forward brand, all without giving up the Dr Bola or Osteon brands.
- **Patients and hospitals:** affordable, credible joint replacement where there was none.

10. Risks and how we manage them

- **Regulatory.** Device importation and registration (NAFDAC) and certification integrity must be watertight. A gate before scaling.
- **Clinical governance across sites.** Roaming introduces variability. Manage with standardised protocols, credentialed surgeons only, clear complication and aftercare ownership per partnership, and outcome tracking at every spoke.
- **Implant quality and liability.** Cheaper must never mean lesser; the transparency that is our strength works against us if an implant fails. Certified supply, outcome monitoring, honest reporting.
- **Anchor concentration.** Kumar is not yet engaged, so early on this leans on Dr Bola. The academy is the mitigation.
- **Brand clarity.** Keep the three brands and their contracts clean, so none is diluted and the commercial lines are unambiguous.
- **Capital and operations.** Inventory, kits and a mobile unit need working capital and disciplined logistics. Model before committing.

11. Roadmap

Phase 0, prove it (now, near free). Dr Bola and OrthoSurplus agree a first commercial arrangement. He uses OrthoSurplus implants in his own Osteon and partner-hospital cases and begins publishing outcomes. The flagship proof, at almost no cost, with the conflict-of-interest governance locked from day one.

Phase 1, pilot the packaged offer. Stand up the roaming unit and the hospital partnership at one or two warm sites; Cedarcrest and Diamed, where Dr Bola already visits, are the natural first spokes. Publish the pilot outcomes and economics.

Phase 2, build the cadre and the second hub. Recruit and train the first associate surgeons through the academy and CadreHealth. Bring Dr Kumar and the Abuja hub in as the model proves out.

Phase 3, scale. Widen hospital partnerships across Nigeria, extend toward Accra and West Africa, launch managed kits, add patient financing, and explore a cross-subsidy so full-pay volume funds access for those who cannot pay.



12. Open decisions

1. **The commercial arrangement itself.** The core question: is Dr Bola engaged on fees and revenue share (a pure commercial deal), or does he also take equity in the arthroplasty-delivery venture, distinct from OrthoSurplus's supply business? This shapes everything downstream. Our lean: a clear commercial deal first (supply pricing plus clinical fees or per-case share), with equity in the clinical venture considered once the pilot proves out.
2. **Brand naming for the clinical arm.** "OrthoSurplus" reads as a supply name and may not suit a clinical, patient-facing service. The roaming and hospital-facing arthroplasty service may need its own name, distinct from both the OrthoSurplus supply brand and the Dr Bola brand. To decide.
3. **Hospital commercial model:** per-case fee vs revenue share vs blended, and the implant economics beneath it.
4. **Kumar and the Abuja hub:** when and how the second anchor comes in.
5. **Regulatory pathway and capital plan,** both of which gate Phase 1.

13. What we need to build this out

- OrthoSurplus's real numbers: implant cost and margin, current members, and the economics of a managed kit.
- Dr Bola's case data, to seed the outcomes record and the flagship proof.
- Agreement on the shape of the commercial arrangement (open decision 1).
- One or two willing pilot hospitals (Cedarcrest and Diamed are the obvious first calls).
- A regulatory and capital review before Phase 1.

The strategy is deliberately built to start almost for free: Phase 0 is just Dr Bola using OrthoSurplus implants and publishing his results under a simple first arrangement, and it compounds from there. The full detail on Dr Bola's own positioning and the Blue Ocean logic sits in the companion documents already delivered.